

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: PHolstPeriod: February 22 to March 23, 2024Report Date: March 24, 2024Coverage: 701/734h (96%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

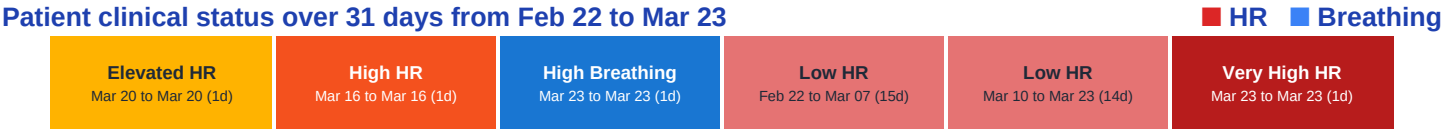
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm



Last 24 Hours

Last 24 Hours: NORMAL				30-Day Tier: YELLOW
Vital	Avg	Min	Max	
Heart Rate	56 bpm	41 bpm	113 bpm	
Breathing	16 brpm	6 brpm	38 brpm	

Last 24 hours: two low heart rate episodes in the morning; one very high heart rate episode in the evening; one high breathing episode in the evening.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 23	9 AM to 10 AM	4h	4	Elevated Breathing	16	6	38	Check SpO2 and lung sounds; assess for fluid overload or early infection
Mar 23	11 PM to 12 AM	1h	1	High Breathing (brief)	30	11	38	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Mar 23	9 AM to 10 AM	2h	2	Low Heart Rate (brief)	43	41	46	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 23	11 PM to 12 AM	1h	1	Very High Heart Rate (brief)	113	111	113	Check temp, hydration, pain; review for arrhythmia or infection

55 episodic events Detected, spanning 3 days total. Conditions: Elevated Heart Rate, High Breathing, High Heart Rate, Low Heart Rate, Very High Heart Rate. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
55	78h	Low Heart Rate	2	96%

Major Findings: Sustained low HR (avg 43, min 40)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 23	11 PM to 12 AM	1h	1	Very High Heart Rate	113	111	113	Check temp, hydration, pain; review for arrhythmia or infection
Mar 16	12 PM to 1 PM	1h	1	High Heart Rate	110	67	119	Check temp, hydration, pain; review for arrhythmia or infection
Mar 23	11 PM to 12 AM	1h	1	High Breathing	30	11	38	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Feb 22	5 PM to 6 PM	56h	38	Low Heart Rate	43	40	77	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 10	6 AM to 8 AM	17h	12	Low Heart Rate	43	40	77	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Mar 20	11 AM to 12 PM	1h	1	Elevated Heart Rate	99	99	107	Check temp, hydration, pain; review for arrhythmia or infection
Mar 09	5 PM to 6 PM	1h	1	Elevated Breathing (brief)	24	12	39	Check SpO2 and lung sounds; assess for fluid overload or early infection

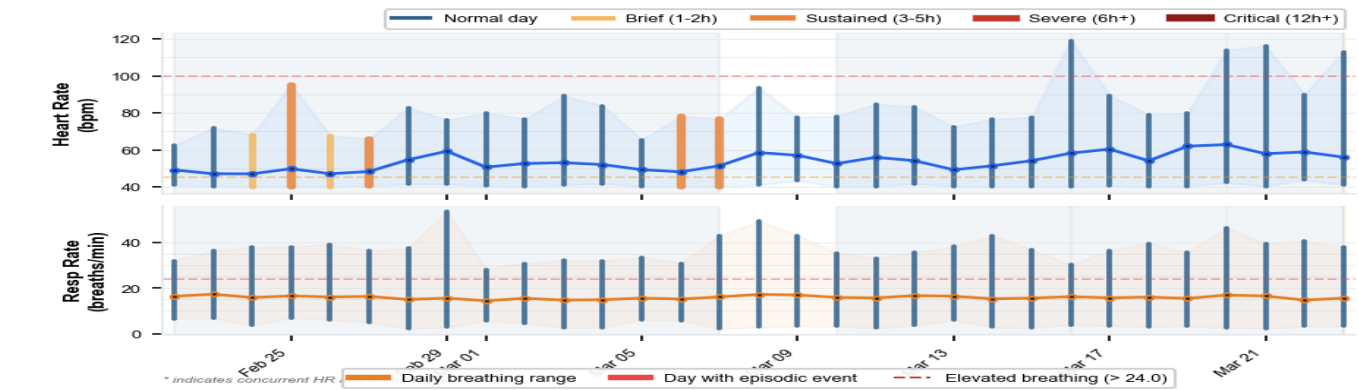
The P5 to P95 heart rate spread was 26 bpm (43 to 70), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Heart Rate (avg 99 bpm, peak 107 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- High Heart Rate (avg 110 bpm, peak 119 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- High Breathing (avg 30 brpm, peak 38 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Heart Rate (avg 113 bpm, peak 113 bpm): Check temp, hydration, pain; review for arrhythmia or infection

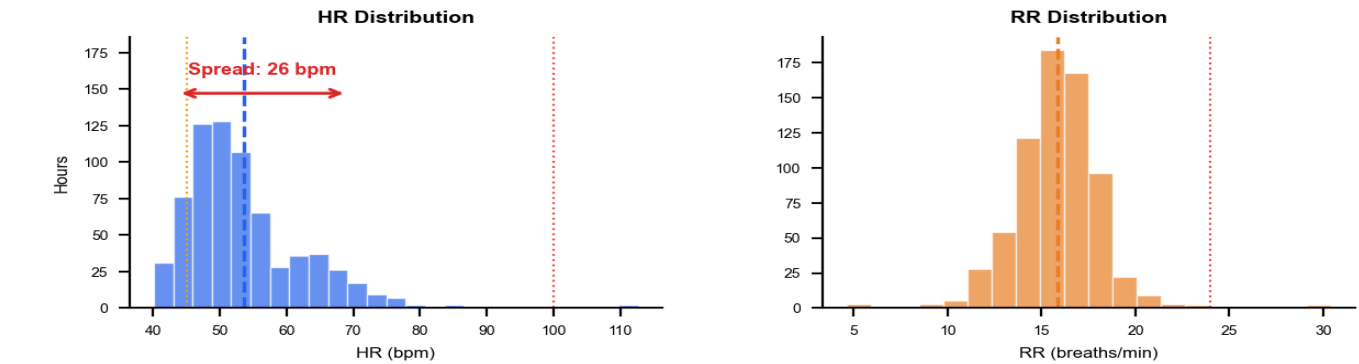
Clinical Pattern Observations:

- 1. **High variability:** HR spread 26 bpm (43 to 69).
- 2. **Clustered pattern:** Episodic events on 6 of 31 days (19%).
- 3. **Nocturnal pattern:** 5 of 6 eligible episodes during evening or night hours.

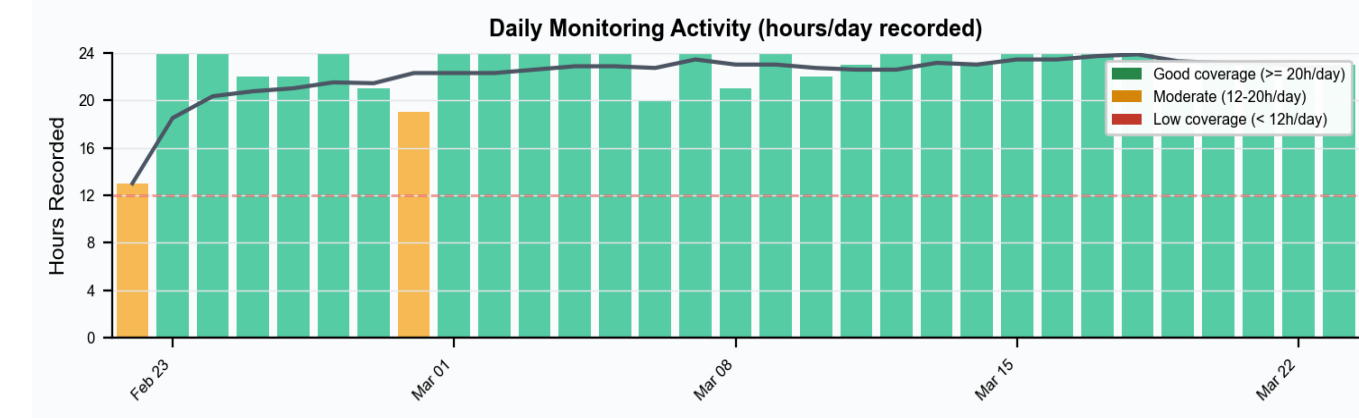


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.